



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سماح ابراهيم احمد محمد خضر

العنوان : 26 ش مسجد الرحمة
الطوابق فيصل

الحى / المدينة : الهرم

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 24-09-2026

رقم البطاقة : 28405202101009 رقم التليفون : 01008214160

العمر (تاريخ الميلاد) : 20-05-1984 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام رنين مغناطيسي علي الفقرات القطنية بالصبغة
قسم العلاج : الاورام
[12771]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سماح ابراهيم احمد محمد خض National ID: 28405202101009	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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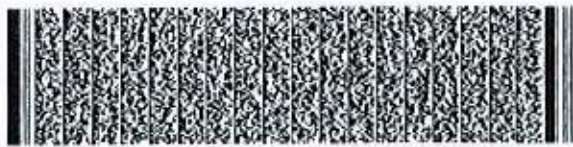
وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced MRI of the lumbar spine
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


 جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 سماح
 ابراهيم احمد محمد خضر
 ٢٦ من مسجد الرحمة الطوايف فيصل
 الهرم - الجيزة
 ١٩٨٤/٠٥/٢١
 ٢٨٤.٥٢.٢١.١٠.٠٩
 1H7289386


 ٢٠٢٠/٠١
 ٢٨٤.٥٢.٢١.١٠.٠٩
 حاصله على ليسانس اداب
 أنثى مسلمة متروجة
 حاتم حسن جميل طي
 البطاقة سارية حتى ٢٠٢٧/٠١/٠٧

 ١


MDT FORM

Name:	سماح ابراهيم احمد محمد	MRN:	12442
Age:	42 Y	ID:	28405202101009
Menopausal Sta	PRE	Phone:*	28405202101009
NRS SCORE:	PS 0	Marital Status:	MARRIED
BMI:	25	Family History:	NAD
Gender		Reason Of MDT:	NEW CASE
CO Morbidity:	NAD	ECOG Performance Status	☒ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4
Surgical History:*	NAD		

Clinical Data:

BX: IDC GII

ER: 2/8 **PR:** 0/8 **HER2:** +3 **KI67:** 15

History:

Sonomammography:

lt breast (BIRADS 4c): upper breast suspicious process
rt breast (BIRADS 1); normal study

Biomarker/Molecular

MRI BREST

Left breast multi-centeric pathological proven malignant process (BIRADS 6)

with ipsilateral indeterminate axillary lymph node.

Left breast LIQ enhancing probably benign looking lesion (BIRADS 3).

Right breast UQQ parenchymal enhancement and multiple scattered enhancing foci with benign kinetics, likely adenotic (BIRADS 3).

Left hepatic lobe closely related well defined lesion.

Mets Workup:

BONE SCAN FREE

CT CAP,, TINY LYTIC LESIONS D5 ,, L4

Recommendations:

MDT Outcome & Plan

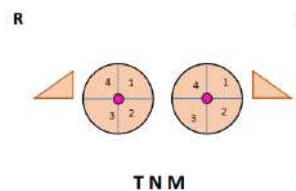
PORTA CATH , ECHO

DYNAMIC MRI LIVER WITH CONTRAST

MRI DORSO-LUMBAR WITH CONTRAST

MEANWHILE START TCHP X 6CYCLES

THEN FOR LT NSM + RECONSTRUCTION+ LN ACCORDINGLY



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د نرمن مصطفى	د محمد النبراوي		

Patient Name	سماح ابراهيم احمد محمد	Case No.	342408
Sex/Age	Female 42 Y	Patient No.	24865
Registered Date	16/7/2026 12:01:45 PM	Referred By Dr.	Interventional Radiology
Validation Date	26/7/2026 01:51:42 PM	Branch	Gustave Roussy
Reporting Date	27/7/2026 09:40:29 AM	Pathology No.	2234/26

Pathology Report

Clinical Data: Left breast mass, BIRADs 4c.

Nature of Specimen: Tru-cut biopsy, collected on 16/07/2026, at 1:51 pm, Cold ischemia time: 20 seconds.

Gross Examination: Three grey white tissue cores, ranging in length 0.3-0.7 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade: 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).
- * Ductal carcinoma in situ: Absent.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Tumor infiltrating lymphocytes (TILs): Mild

Diagnosis:

Left Breast Mass; Tru-Cut Biopsy:

- Invasive Duct Carcinoma, Grade 2.
- For Further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



غرفة رقم (4) - الطابق الاول - مبني رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misy El Qadema - Cairo, Egypt

Patient Name	سماح ابراهيم احمد محمد	Case No.	344663
Sex/Age	Female 42 Y	Patient No.	24865
Registered Date	28/7/2026 03:05:45 PM	Referred By Dr.	Interventional Radiology
Validation Date	01/8/2026 09:21:52 AM	Branch	Gustave Roussy
Reporting Date	02/8/2026 02:35:01 PM	Pathology No.	2234/26

Biomarker report

Clinical Data: Left breast 12 O'clock mass (BIRADs 4c) diagnosed "Invasive duct carcinoma grade 2" (Positive family history).

Nature of Specimen: Tru-cut biopsy, collected on 16/07/2026, at 1:51 pm. Cold ischemia time is 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (All controls showed appropriate reactivity):
- ER: Weak nuclear reaction in less than 1% of tumor cells (score 2/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 15%.

Conclusion:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 2.

IPT:

- ER: Negative (score 2/8).



Patient Name	سماح ابراهيم احمد محمد	Case No.	344663
Sex/Age	Female 42 Y	Patient No.	24865
Registered Date	28/7/2026 03:05:45 PM	Referred By Dr.	Interventional Radiology
Validation Date	01/8/2026 09:21:52 AM	Branch	Gustave Roussy
Reporting Date	02/8/2026 02:35:01 PM	Pathology No.	2234/26

- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67%: 15%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt

Name: Mrs. Samah Ibrahim Ahmed Mohammed

Date: 6/8/2026

ID:1118023203

Acc.:319017855

Referring Prof. Dr. Mohammed El-Nabarawy

Tc-99m MDP whole body bone scan

Technique:

- Whole body images were acquired 180 minutes after IV injection of ~15 mCi of the tracer.

Findings:

- Uniform tracer uptake all over the surveyed skeleton is noted with no evidence of any active spots.
- Degenerative changes of the spine are seen.
- Arthritic knee changes are noted.
- Both kidneys are visualized.

Opinion:

- No scintigraphic evidence of metastatic osseous deposits.

N. B.: Purely lytic lesions cannot be resolved on bone scan and better seen in CT/MRI.

Best Regards;

Prof. Dr. Ahmed Kandeel
MD Nuclear Medicine

Typed: A. El-Sayed. 08/08/26 10:38:43 AM

Patient's Name: Samah Ibrahim Ahmed Mohammed Khidr
Date: 2026-08-01
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA.

I- CHEST:

- Few tiny bilateral scattered pulmonary nodules measuring 2-3 mm in diameter.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the uterus.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Two tiny D5 and L4 lytic foci.

Opinion:

- ❖ Few tiny bilateral scattered pulmonary nodules likely inflammatory for follow up.
- ❖ No pulmonary or hepatic metastatic lesions.
- ❖ Two tiny D5 and L4 vertebral bodies lytic foci for bone scan.

Dr.sherif Wagdy

Dr.sherif Wagdy

Patient's Name: Samah Ibrahim Ahmed Mohammed Khidr
Date: 2026-07-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 40-year- old female with positive family history (sister), complaining of right palpable lump of few days duration.

Findings:

Left breast:

- Scattered fibroglandular densities. (ACR b)
- UOQ, posterior and middle thirds, 4 cm from nipple, segmental clustered microcalcifications seen involving an area measuring 10.3 x 9.5 x 8.7 cm with surrounding distortion. within such area at the middle third; irregular speculated mass 13 x 10 mm.
- Ultrasound demonstrated 11-2 o'clock, posterior & middle thirds, prominent ducts with soft-tissue echogenicity within reaching 3 mm in caliber with surrounding areas of altered parenchyma mounting to mass formation at 1 o'clock, 5 cm from the nipple hypochoic ill-defined irregular anti-parallel mass lesion is noted measuring 26 x 18 mm.
- Non-specific axillary lymph nodes. No pathological infra r supra-clavicular lymph nodes

Right breast:

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Left breast (BIRADS 4c): upper breast suspicious process as described ?? calcified DCIS.

Right breast (BIRADS 1): normal study

Conclusion:-

Left breast: for US-guided Tru-cut biopsy and histopathological correlation.

***Much obliged,
Dr. Amal Hegazi, M.Sc.***

PATIENT NAME: SAMAH IBRAHIM AHMED MOHAMMED

GENDER: Female

DATE : 04-Aug-2026 12:26 PM

PATIENT ID: 10126283664

AGE: 42 y

ORDER:

3984074,3984641,3984644

Dynamic Contrast Enhanced MRI Study of the Breasts

Clinical summary:

- Left breast pathologically proved IDC.

Pulse Sequences:

- Pre-contrast: axial T1,T2,STIR and sagittal T2 WIs.
- Post contrast: axial VIBE and Subtraction WIs.
- DWIs and ADC map.

Findings:

The breasts are heterogeneously dense, showing bilateral moderate asymmetrical background parenchymal enhancement.

LEFT BREAST:

- Multiple irregular lesions are noted mainly at the upper quadrants, eliciting low signal on T1 and T2 and heterogeneous intermediate/bright signal on STIR, with restricted diffusion
- On post contrast series, the lesions shows heterogeneous post-contrast enhancement with heterogeneous NME seen within the intervening parenchyma. On plotting time signal intensity curve, type III curve is displayed.
- The largest lesion seen at 2:00, measuring **4.2 x 3.3cm** in diameter. The nearest lesion to skin is seen at 2/3 O'clock (zone C), being **8mm** from lateral skin, the most posterior lesion being **5mm** from pectoralis muscle.
- Multiple enhancing nodules/foci are noted scattered through parenchyma, the most anterior one is seen retroareolar; few seen subdermal.
- The whole pathological enhancing process (mass & NME) occupying an area measuring **13 x 7x 6cm** in maximum AP, CC and TS dimensions respectively.
- **Complementary mammography and Tomosynthesis images** revealed multiple irregular lesions along with multiple groups of pleomorphic microcalcifications are noted mainly at the upper quadrants and extending to retroareolar region.
- Left LIQ circumscribed lobulated lesion is noted, eliciting low signal on T1 and

PATIENT: SAMAH IBRAHIM AHMED MOHAMMED, 10126283664, Female, 42 y

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مركز بهية الهرم
4 شارع علوية نول شارع الهرم بجوار داري , الجيزة

مستشفى بهية الشيخ زايد
٤٠ الحي الأول - الشيخ زايد أمام كلية الهندسة

T2 and bright signal on STIR, showing post contrast enhancement with non-enhancing internal septations, with type I curve, measuring **1.5 x 0.9cm** in diameter, being 4cm from nipple. 2nd Look ultrasound revealed corresponding circumscribed lobulated hypoechoic lesion, located at 8:00.

- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Prominent axillary lymph nodes are noted at level I, 2nd Look ultrasound revealed corresponding lymph node with non-uniform cortical thickening, measuring about 3.2mm in thickness.

RIGHT BREAST:

- On post-contrast series, UOQ faint parenchymal enhancement is noted along with multiple scattered enhancing foci showing type I curve. 2nd Look ultrasound revealed no correlate.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

N.B. A rather well defined lesion is noted closely related to the inferior aspect of left hepatic lobe, eliciting low T1, bright T2 & STIR signal, measuring about 2.7x2cm in diameter.

Opinion:

- Left breast multi-centeric pathological proven malignant process (**BIRADS 6**) with ipsilateral indeterminate axillary lymph node.
- Left breast LIQ enhancing probably benign looking lesion (**BIRADS 3**).
- Right breast UOQ parenchymal enhancement and multiple scattered enhancing foci with benign kinetics, likely adenotic (**BIRADS 3**).
- Left hepatic lobe closely related well defined lesion, for further assessment with dedicated MRI dynamic liver study.

Reported By,
Dr Lobna KHALED.MD
Dr. EMAN FAKER, MD

06-Aug-2026 01:46 PM

PATIENT: SAMAH IBRAHIM AHMED MOHAMMED, 10126283664, Female, 42 y

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مركز بهية الهرم
4 شارع علوية نول شارع الهرم بجوار داري , الجيزة
مستشفى بهية الشيخ زايد
ع. الحي الأول - الشيخ زايد أمام كلية الهندسة



PATIENT NAME: SAMAH IBRAHIM AHMED MOHAMMED

PATIENT ID: 10126283664

GENDER: Female

AGE: 42 y

DATE : 05-Sep-2026 10:12 AM

ORDER: 4021208

MRI OF THE ABDOMEN WITH DYNAMIC STUDY OF THE LIVER

CLINICAL BACKGROUND:

- Pathologically proven case of left breast cancer.

MRI FINDINGS:

- Enlarged liver measuring about 17 cm in the right MCL harboring few hepatic focal lesions as follows:
 - Two small focal lesion seen at segment V/VIII (11x12mm) as well as segment II (19x23mm) showing low T1 and high T2 signal with early discontinuous peripheral post contrast enhancement with progressive filling in, one of them (at segment V/VIII) shows mild peripheral diffusion restriction.
 - Three small focal lesions seen at segment IVa (6x7mm) as well as segment VI (the larger of two measures 12x10mm) showing low T1 and high T2 signal with early post contrast enhancement showing a fainter enhancement at the portal and delayed phases, they show no related diffusion restriction.
 - Patent portal vein and its branches. No dilated biliary radicles.
 - Normal size, shape and signal intensity pattern of the spleen, pancreas and both kidneys.
 - No ascites.
 - No evidence of abdominal soft tissue mass lesions or enlarged lymph node.

OPINION:

- Enlarged liver, showing:

1. Three small focal lesions seen at segment IVa as well as segment VI, likely flash filling hemangioma, for close follow-up.
2. Two small focal lesion seen at segment V/VIII as well as segment II benign looking hepatic focal lesions show enhancement pattern in keeping with cavernous hemangiomas.

PATIENT: SAMAH IBRAHIM AHMED MOHAMMED, 10126283664, Female, 42 y

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مركز بيهية
أشعة تشخيصية
مستشفى بيهية التخصصي
ع. النجف الأولى - السبع أحياء - كركوك - العراق



PATIENT NAME: SAMAH IBRAHIM AHMED MOHAMMED

GENDER: Female

DATE : 05-Sep-2026 10:12 AM

PATIENT ID: 10126283664

AGE: 42 y

ORDER: 4021208

MRI OF THE ABDOMEN WITH DYNAMIC STUDY OF THE LIVER

CLINICAL BACKGROUND:

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MRI FINDINGS:

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2. Two small focal lesion seen at segment V/VIII as well as segment II benign looking hepatic focal lesions show enhancement pattern in keeping with cavernous hemangiomas.



Reported By,
Dr , Yomna Ali, MSc
Dr. Shereen AbdelRaouf, MD

06-Sep-2026 06:09 PM

PATIENT: SAMAH IBRAHIM AHMED MOHAMMED, 10126283664, Female, 42 y

Report version 2, 06-Sep-2026 06:09 PM

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مركز بحوث الدم
4 شارع طوبه بول شارع الدم بحور دوى ، الجدة
مستشفى بحوث الدم زائل
ع. حقى الاول - الشارع زائل عام كلية الطب

